THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

From Reset *to Rest*

A gentle night-waking reduction protocol

For families ready to move from stabilised sleep to more consolidated rest, without cry-it-out.

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How to use this guide

If you're here, you've likely restored some calm to bedtime, but night wakings remain.

This guide helps you reduce those wakings gradually and respectfully, while protecting the emotional safety you've worked hard to rebuild. It's a framework, not a rigid programme. By moving slowly and holding steady, your child's sleep will consolidate.

The four steps

1
Confirm readiness

Check your child is ready before you reduce anything.

PAGE 3

2
Identify the driver

Work out what is keeping the wake-ups going.

PAGE 4

3
Choose your anchor

One consistent overnight response, held for three nights.

PAGE 5

4
Reduce one variable

Small, respectful changes, one at a time.

PAGE 6

YOU'LL LEARN TO

- Assess whether your child is ready for reduction
- Understand what is driving the waking
- Reduce wakings gradually and respectfully
- Keep your child feeling safe while encouraging longer stretches

BEFORE YOU START

- Watch the video walkthrough once, ideally with your partner
- Start the tracker on page 9 tonight, before you change anything
- Agree that every adult will respond the same way overnight

General guidance, not medical advice. Check with your child's doctor before reducing night feeds, pause if your child becomes unwell, and always follow safe sleep guidance: alone, always starting on their back, in a cot or crib with only a fitted sheet.

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Confirm readiness

Reducing wakings before your child is ready often leads to escalation. Use this checklist first.

SLEEP PRESSURE MARKERS

- [ ] Bedtime happens within a consistent 30-minute window
- [ ] Daytime sleep is age-appropriate
- [ ] No frequent overtired meltdowns before bed
- [ ] The first stretch of night sleep is at least 2–3 hours

REGULATION MARKERS

- [ ] Bedtime is calm more often than chaotic
- [ ] Your child can settle with support, without escalating straight away
- [ ] You've kept overnight responses consistent for 3–5 days

What your answers mean

MOST BOXES TICKED

Your child is ready. Move on to Step 2 and identify what is keeping the wake-ups going.

SEVERAL BOXES UNTICKED

Pause. Return to stabilisation for a few more days with the free Gentle Sleep Reset, then check again. This protects your progress.

NOTES

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Identify the driver

Night wakings persist for different reasons. Find the main one before you adjust anything.

DRIVER

Habit loop

- Waking at predictable times each night
- The same sequence every time
- An immediate expectation of a specific response

DRIVER

Timing imbalance

- Short naps alongside frequent waking
- Very long wake windows before bed
- Early rising before 5:30am

DRIVER

Regulation dependency

- Can't fall back asleep without one specific support
- Strong physical reliance: rocking, feeding, bouncing
- Escalation when support is reduced suddenly

If more than one fits, choose the one you see on most nights. You can come back to the others later.

OUR PRIMARY DRIVER

☐ Habit loop ☐ Timing imbalance ☐ Regulation dependency

WHAT WE NOTICED

> **Timing imbalance?** Wake windows and naps are often the quickest place to start. The troubleshooting table on page 8 shows what to check first.

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Choose your anchor

Children settle when patterns are predictable. Choose one consistent overnight response.

	ANCHOR RESPONSE	WHAT IT LOOKS LIKE
☐	**Feeding, with gradual duration reduction**	Shorten the feed a little at a time, rather than stopping it.
☐	**Physical presence, with reduced intensity**	Stay close, with less patting, rocking or holding over time.
☐	**Verbal reassurance, with a timed pause**	A short pause, then your calm phrase from beside the cot.
☐	**A combination approach, defined clearly**	Allowed, as long as every adult does exactly the same thing.

Commit for three nights before reassessing. No rotating strategies: changing the response night to night is what keeps children waking to find out which one they'll get.

Your overnight phrase

Consistency reduces negotiation. Choose one calming phrase and repeat it every time.

"You're safe. It's still sleep time."

"It's night-time. I'm here. It's time to sleep."

"Shhh, it's night-time."

OURS, WORD FOR WORD

_____

_____

AVOID

- Lengthy explanations
- Negotiating
- New words each night

Your calm tone matters more than your words.

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Reduce one variable at a time

Choose one part of your anchor response to reduce, a little at a time. Leave everything else the same.

☐	VARIABLE	WHAT YOU'RE REDUCING
1	**Feeding duration**	How long each night feed lasts
2	**Rocking intensity**	How much movement you give, and for how long
3	**Time before responding**	The short pause before you go in
4	**Physical proximity**	How close you are while they settle
5	**Length of intervention**	How long you stay before stepping back

The four-night rhythm

NIGHTS 1–3
Make one small adjustment and hold it

→

NIGHT 4
Assess, using your tracker

→

Progress? Continue

Escalation? Reassess the driver

PROGRESS LOOKS LIKE

- Slightly shorter wakes
- Less intensity
- A longer first stretch

Not perfection. Any progress, no matter how small, is a win.

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Night feeds & when to pause

If night feeding is part of the pattern, reduce it gently. Never stop abruptly unless your child's doctor has cleared it.

OPTION A

Shorten

Reduce feeding duration by 1–2 minutes every 2–3 nights.

OPTION B

Space out

Gradually increase the interval between feeds.

OPTION C

Earlier first

Shift toward consolidating the earlier part of the night first.

Watch for genuine hunger cues, illness and growth spurts. **When in doubt, pause reduction.**

When to pause the whole plan

- Illness returns
- Travel
- A major developmental leap
- Family stress increases significantly

Return to stabilisation first. Then resume.

Go back to your bedtime window and anchor response for a few days, then pick up at the step you'd reached.

Feed reduction log

NIGHT	FEED TIME	MINUTES OR ML / OZ	HOW THEY RESETTLED
1			
2			
3			
4			
5			

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Troubleshooting

After 7–10 nights, if you're still seeing one of these patterns, other variables may be involved.

PATTERN	USUALLY SUGGESTS	CHECK FIRST	SUGGESTED ADJUSTMENT
Wakings every 45–90 minutes	Overtiredness or sleep-cycle dependency	Total wake windows, nap structure, bedtime timing	A slightly earlier bedtime, or a shorter last wake window
Split nights	Under-tiredness, or too much daytime sleep	Total nap duration, afternoon sleep, bedtime	Reduce daytime sleep, or move bedtime later
Early rising before 5:30am	Circadian shift or sleep-pressure imbalance	First nap timing, bedtime timing, total day sleep	Adjust nap distribution and protect the first wake window
Intense escalation when support is reduced	Change has moved faster than regulation can manage	Amount reduced, pace of change, baseline settling	Return to stabilisation, then reduce more gradually
Nap refusal	Sleep-pressure mismatch or overtiredness	Morning wake time, nap timing, stimulation	Rebalance the day before pushing overnight change
Strong sensitivity traits	Higher support needs; lower tolerance for abrupt shifts	Sensory load, predictability, response consistency	Slower reduction, more reassurance, temperament-led pacing

You haven't failed. You may simply need nuance.

Complex sleep patterns benefit from tailored timing adjustments, a feeding review and temperament-informed planning. This is often where a personalised assessment becomes valuable: see page 10.

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Your 7-night tracker

Fill it in each morning. Patterns become visible when written down, and clarity reduces anxiety.

NIGHT	BEDTIME	WAKINGS (#)	LENGTH OF WAKING	RESPONSE USED	INTENSITY (1–5)
1					
2					
3					
4 assess					
5					
6					
7					

NIGHT 4 ASSESSMENT

☐ Progress → continue ☐ Escalation → reassess the driver

_____

OUR VARIABLE & THE STEP WE'RE ON

_____

_____

Intensity: 1 = brief stir, settles quickly · 3 = needs your anchor response · 5 = very upset or escalating. Print a fresh copy for each new step.

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Next steps

If you're still seeing persistent fragmentation after working this framework, you may simply need a plan built around your child.

SOME CHILDREN

- Wake because of complex timing imbalances
- Have layered sleep associations
- Are highly sensitive or neurodivergent
- Need their feeding strategy refined
- Rise early for reasons unrelated to habit

These children benefit from individualised guidance. Personalised support can help you move forward more efficiently, without undoing the connection you've protected.

NOT SURE WHAT YOU NEED?

Book a free 15-minute sleep assessment call at thyme4sleep.com. There's no obligation.

PERSONALISED 1:1 SUPPORT

A plan written for your child, including naps and schedule. See thyme4sleep.com/pricing, and mention this guide.

Move slowly. Hold steady. Trust consistency.

Sleep consolidates when safety and structure align.

— Georgia

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